DELIVERED PRIVATELY TO

Dr. Chima Oti, Managing Director, and the Board
Dennis Ashley Medical Clinic & Endoscopy Suites
Landmark Towers, 5B Water Corporation Road, Oniru, Victoria Island, Lagos

25 September 2026
Ref: MZ-ENG-DAMC-2026-R2

REVISED ENGAGEMENT PROPOSAL

The plan for Dennis Ashley: what we bring, what we fill, and what we will ask for next

Dear Dr. Oti and Members of the Board,

Since our last exchange we have walked the facility and we have assembled the bench. This revision replaces the proposal of 1 September. It is shorter, and it is written to be decided on rather than studied: **the plan below does not depend on an audit**. We still want your numbers, and we set out at the end exactly what we would need, but that is to build the financial model and the growth plan, and to fix the baseline we are measured against. It is not a gate on the decision.

1 · The plan, in one paragraph

Dennis Ashley remains **100% yours**: ownership, brand, premises and clinical governance unchanged. Mezo takes on the day-to-day operations and runs the clinic on our operating platform, to targets agreed with the board and reported weekly. What is new in this revision is the substance behind the growth: **Mezo arrives with a bench of specialist practices**, already operating, already carrying patients, who will run their clinics and day-case lists out of Oniru. Your rooms and your endoscopy suite stop waiting for demand to find them. Our fee remains **7.5% of monthly collected revenue**, plus a performance incentive on agreed growth milestones, so we are paid on cash that arrives and our upside is tied to yours.

2 · What we found on the visit

We have now spent time in the building rather than only in the numbers, and two things are worth putting in writing.

The asset is ahead of the trading. The address, the fit-out, the endoscopy suite and the name carry more weight in this market than the current level of activity reflects. Nothing about the building is holding the practice back. That is the whole reason we are here: the gap is demand and operating

rhythm, not bricks, equipment or clinical quality. It is also why we can propose a plan that needs no capital from either side.

There is no demand engine behind the practice. A premium clinic at this address should not be waiting for patients to find it, and at present very little is being done to make sure they do. There is a presence on social media; there is not a marketing programme, a referral pipeline, a published price card or a booking route that a stranger can use. That is a solvable problem, and it is largely what section 8's marketing budget and the demand engine in section 3 are for.

Neither observation is a criticism of the board. It is what happens in any practice running below its capacity, where nobody has been given the brief to close the gap. Giving one party that brief, and holding them to it weekly, is precisely what this engagement is.

3 · What we bring is traction, not intentions

Every management proposal promises to go and find demand. We are not asking the board to take that on trust. **The practices below already exist and already see patients.** They are the bench we would bring to Oniru, and each of them needs an Island address with a consulting room, a treatment space or an endoscopy list. Filling your week from here is a scheduling exercise, not a business development exercise.

Vehicle	What it is today	What it brings to Oniru
Paras Orthocare Dr. Rajeev Kumar	An operating orthopaedic and arthroscopy practice.	Orthopaedic consultation and arthroscopy clinics, and the musculoskeletal caseload that comes with them.
Osteon Clinics Dr. Bolarinwa Akinola, FRCS (Tr & Orth)	An orthopaedic centre running its own theatres and surgical lists.	Island consultation, pre-assessment and post-operative clinics. Surgery runs at Osteon's theatres; the patient relationship sits at Oniru.
Nelia Dr. Folake Kofo Idowu	A women's health and regenerative medicine practice.	Women's health clinics and treatment-room procedures, the natural companion to a wellness practice with a corporate book.
Cooked Indoors Medical Nutrition The Balance Clinic · Sito Metabolic Health Clinic	Two live clinics taking bookings online today, built around medical weight management and specialist diabetes and metabolic care.	Programme-based patients rather than single visits, and a standing pipeline of people who need scopes, screening and chronic-care follow-up.
GetSlim Bariatric & General Surgery Clinics	Bariatric and general surgery clinics with their own endoscopy lists.	Surgical clinics, and gastroscopy and colonoscopy lists into your suite. Bariatric workup requires endoscopy by protocol, so this flow is clinical, not persuaded.

Vehicle	What it is today	What it brings to Oniru
Haven Paediatrics	An established Ikeja practice with a paediatric and neonatal book, actively seeking an Island extension.	Paediatric sessions at Oniru, and the one thing a corporate wellness tender usually has to decline: cover for employees' children.

Behind them sits the rest of the machine: the network front door at **mezohealth.com** with published prices and online booking, search and content marketing run centrally, the corporate channel, and the diaspora channel where families abroad fund care for parents at home. **Being added now:** urology, which sits naturally beside a corporate executive-health book, and beyond it ENT, dermatology with minor surgery, pain medicine, cardiology with echo, and sleep studies. Each is selected on one test: it runs in a consulting room or a day-case slot, and the patient goes home the same day.

> None of the above is a recruitment plan, a hiring budget or a marketing hypothesis. It is **demand that already exists, redirected to an address with the room to take it**. That is the whole of the growth case, and it is why we are comfortable being paid on what is collected.

4 · What that does to the week

Today the clinic's rooms and suite work to the rhythm of one practice. Under this plan they work to the rhythm of nine. The grid below is the shape at maturity rather than month one, and it is illustrative: the real one is built around your own lists and the Administrator's timetable. Year one reaches roughly half of it, which is what the attached build-up costs out.

	Consulting rooms	Endoscopy suite	Treatment room
Monday	Dennis Ashley general and wellness · Nelia women's health	Corporate screening workup	Infusions and minor procedures
Tuesday	Osteon orthopaedics · Balance Clinic weight management	**Endoscopy list**	Injections and regenerative procedures
Wednesday	Dennis Ashley · urology (from launch of that bench)	Corporate and executive health	Minor procedures
Thursday	Paras Orthocare arthroscopy clinic · GetSlim bariatric and general surgery	**Endoscopy list**	Pre-assessment and post-op review
Friday	Dennis Ashley · Sito metabolic and diabetes	Flexible list	Infusions and dressings

	Consulting rooms	Endoscopy suite	Treatment room
Saturday	Haven Paediatrics · executive health	**Endoscopy list**	Screening and phlebotomy

What that week is worth, month by month, is set out in the accompanying **Revenue Build-up** (Ref: MZ-ENG-DAMC-2026-BU). It is built from sessions, slots and lists rather than percentages, it assumes no capital and no price rises, and it holds the clinic's existing business flat throughout.

> **Why the suite fills by necessity, not persuasion.** Bariatric assessment requires endoscopy by protocol. Metabolic and weight-management patients present the same indications. Add network gastrointestinal referrals and corporate screening findings, and the suite moves from episodic to a standing weekly rhythm, against equipment that is already paid for. Day-case endoscopy is among the highest-margin work in outpatient medicine.

5 · The part we would rather say now than later: space

The plan in the attached build-up fits inside the clinic exactly as it stands: at the end of year one it uses a little over 40% of the sellable room sessions, and half the building is still idle. On that trajectory the rooms become the binding constraint **during year two, not year one**. We are telling the board now rather than after the fact: when the week does fill, we will come back and ask for more space, a third and fourth consulting room, a second procedure room and proper recovery capacity, whether that is taken within Landmark Towers or adjacent to it. When we do, it will arrive as a costed proposal with the utilisation figures that justify it, for the board to approve or decline. No capital commitment is asked of Dennis Ashley in this engagement, and none is implied by signing it.

6 · Governance: the board's role, and ours

- **Clinical governance stays with the board.** Protocols, standards and consultant appointments remain yours, and every bench practice works to your standards on your premises. Clinical priority outranks commercial convenience, always.
- **The board is our principal.** Weekly performance reports, monthly reviews, and the annual plan and budget for your approval. Directors in Lagos and London see the same live dashboards at the same time.
- **Mezo holds the day-to-day mandate.** Within the agreed budget, standards and plan, operational decisions sit with the Mezo team. We can only answer for results if we are running the floor. Anything outside the agreed frame comes back to the board.
- **Structured cleanly.** All patient billing runs through the clinic. A bench practitioner is paid a professional-fee share of the work they personally do, and everything around it, the diagnostics, the pharmacy, the treatment room and the suite, is the clinic's. Mezo is paid a management fee by the clinic. Nobody is paid for a referral, which keeps the whole arrangement inside MDCN ethics and your own governance.

7 · The terms, plainly

Term	Position
Management fee	**7.5% of monthly collected revenue**, invoiced monthly in arrears. Collected, not billed: we are paid on cash that arrives.
Performance incentive	Earned on growth milestones measured against an agreed baseline, and written into the Management Agreement. Our upside is tied to yours.
Bench revenue	The clinic bills the patient for everything, through the platform and visible to the board in real time. The bench practitioner is paid a **professional-fee share** of their own consultations and procedures, typically half. The diagnostics, pharmacy, treatment room and endoscopy revenue those visits generate stay wholly with the clinic, which is where most of the growth in the attached build-up sits.
Ownership & brand	Unchanged. Dennis Ashley remains wholly yours; how the name is carried is the board's decision.
Term & exit	Initial term and notice period agreed at signing; either side can exit on notice. Data, standards and improvements remain with the clinic.
Costs	Mezo funds its own management team, the platform, the bench relationships and the network demand engine. The clinic continues to carry its operating costs, including existing payroll, and commits an agreed marketing budget. Set out in full at section 8.

8 · Who does what, and who funds what

The division is simple. **Mezo brings the capability; the clinic carries its own operating costs**, as it does today. We are not asking Dennis Ashley to fund Mezo, and Mezo is not subsidising the clinic's running costs. Each side puts in what it is best placed to put in.

Mezo provides, at our cost	Dennis Ashley funds, as it does today
The management team and its time: the general manager, the operating discipline and the weekly reporting.	**The existing staff.** The team currently hired stays on the clinic's payroll, unchanged. We manage them, you employ them.
The operating platform, HospitlOS, with billing, booking, claims tracking and live dashboards for every director.	**Premises and running costs:** rent, service charge, utilities, licences, indemnity and statutory costs.

Mezo provides, at our cost	Dennis Ashley funds, as it does today
Financial management: daily collections reconciliation, payer claims to settlement, management accounts, budget and controls.	**Consumables, pharmacy stock and laboratory outsourcing**, bought on network terms and, where suppliers allow, held on **vendor-managed inventory**: the supplier owns the stock on your shelf until it is used, so the clinic stops tying up cash in inventory.
Procurement: negotiation with suppliers at network volumes, the VMI arrangements, and the tender discipline behind them.	**An agreed monthly marketing budget:** content creation and paid media. This is the one new line we would ask the board to commit. The attached build-up carries **N750,000 a month** from month one as a working figure, and the final number is set with you, not by us.
The bench and the demand engine: the specialist practices, the network front door at mezohealth.com, search and content marketing run centrally, the corporate channel and the diaspora channel.	Any capital project, if the board approves one. None is required to start, and none is implied by signing.

> **On the marketing budget, plainly.** Two things happen at once when the bench arrives, and both need to be seen. The market has to know that orthopaedics, women's health, bariatric surgery, paediatrics and metabolic medicine now sit at this address, and **the Dennis Ashley brand itself has to be lifted to match what it will be offering**. That means professional content shot at the clinic, a refreshed presence, and paid media behind the services with the shortest path to a booking. Mezo runs the work and the agencies; the spend sits with the clinic, against a budget the board agrees and reviews quarterly on what it returned in leads, bookings and revenue. We would rather ask for it openly at the start than discover the plan stalling for want of it. As section 2 records, a presence on social media is not a demand engine, and the gap between the two is where a great deal of this clinic's missing revenue sits.

9 · What we need from you, and why it does not hold anything up

To build the financial model and the growth plan, and to fix the baseline every later target is measured against, we would need the following. A twelve-month lookback is ideal; less is workable, and the nearest equivalent of anything on this list is enough.

- **Revenue and collections** for the last twelve months, split by service line and by payer, with the cash versus payer mix.
- **Operating costs:** management accounts or P&L, rent and service charge, utilities, payroll by role, and the main supplier lines (laboratory outsourcing, consumables, pharmacy).
- **The price card** as it stands today, including any packages already in market.

- **The payer panel:** the eleven payers with their tariff schedules, and claims submitted, paid, rejected and outstanding with ageing.
- **The corporate book:** contracts, covered headcount, per-employee terms and renewal dates.
- **Activity and people:** consultation and endoscopy volumes by month, the current session timetable, the staff list and rota.

> All of it sits under a **mutual NDA**, in a secure folder you control, used only for this purpose and returned or destroyed if we do not proceed. No patient-identifiable data is requested or required. And to be explicit: **the plan above stands on its own**. Your numbers refine the model and set the baseline; they do not change what we are proposing to do.

10 · Next step

One meeting, in Lagos or by video for the London-based directors, to walk the board through this and answer what it raises. If the board is content, we sign, and the first bench clinics can be sitting in your rooms within weeks rather than quarters. The NDA is drafted and attached, and the secure folder goes up the same day you ask for it.

Dennis Ashley is the right address, with the right brand, in the right market. What it has been missing is volume, and volume is precisely what we are bringing through the door. It would be a privilege to build on what you have created.

With respect and warm regards,

Dr. Jubril Adebowale Odulana

Founding Partner & Managing Director · Mezo, a Consult For Africa vehicle

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